FLUDROCORTISONE

Addison's replacement

Hypokalaemia, oedema

CHEMICAL NAME 9-fluoro-11β,17,21-trihydroxypregn-4-ene-3,20-dione 21-acetate
CLASS Synthetic mineralocorticoid
TRADE NAMES Florinef · Astonin H · Fludrocortisone acetate

THERAPEUTIC USES / INDICATIONS

- Primary adrenal insufficiency (Addison's); mineralocorticoid replacement with hydrocortisone.
- Congenital adrenal hyperplasia (salt-wasting).
- Postural hypotension / orthostatic intolerance (off-label).

ADMINISTRATION

- **PO** once daily in the morning.

DOSAGES

- **Addison's:** 50–200 mcg once daily (usually 100 mcg).
- **Postural hypotension:** 100–400 mcg daily.
- **Infants (CAH):** 50–300 mcg daily (plus salt).

PHARMACOKINETICS

- **Onset:** hours. **Duration:** 18–36 h.
- Metabolised in liver.
- Excreted in urine. **t½:** 3.5 h (biological 18–36 h).

PHARMACODYNAMICS

- Potent **mineralocorticoid** (~125× hydrocortisone) → ↑ Na^+ and water reabsorption, ↑ K^+ and H^+ excretion in distal tubule → ↑ BP, plasma volume.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Fluid retention, oedema.
- Hypertension.
- Headache.

ADVERSE EFFECTS (SERIOUS)

- Hypokalaemia, hypokalaemic alkalosis.
- Heart failure.
- Muscle weakness.

CONTRAINDICATIONS & CAUTIONS

- Systemic fungal infection.
- **Caution:** hypertension, heart failure, renal impairment, hypokalaemia.

KEY INTERACTIONS

- **Diuretics, amphotericin, laxatives:** ↑ hypokalaemia.
- **Digoxin:** toxicity with hypokalaemia.
- **Enzyme inducers:** ↓ effect.

NURSING MANAGEMENT

- Monitor BP (lying/standing), weight, oedema, K^+, Na^+.
- Adjust dose to postural BP and renin.
- Educate with hydrocortisone sick-day rules (fludrocortisone not doubled).

PATIENT EDUCATION

- Take every morning with hydrocortisone.
- Report swelling, headache, muscle weakness or dizziness.
- Do not double this tablet during illness (only hydrocortisone).

KEY POINTS

- Mineralocorticoid replacement in Addison's.
- Oedema, hypertension, hypokalaemia.
- 50–200 mcg daily.
- Monitor BP and K^+.